DSM-001

Psychosis lasting at least 1 day but under 1 month, with full return to usual functioning?

DSM-002

Schizophrenia-type symptoms lasting 1 month to under 6 months?

DSM-003

Minimum duration for schizophrenia?

DSM-004

The five symptom domains that define psychotic disorders?

DSM-005

Examples of negative symptoms, and why they matter?

DSM-006

Psychotic symptoms that occur only during mood episodes?

DSM-007

What separates schizoaffective disorder from a mood disorder with psychotic features?

DSM-008

Delusional disorder versus schizophrenia?

DSM-002

Schizophreniform disorder. Labeled 'provisional' while the person is still ill. A decline in functioning is not required.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-001

Brief psychotic disorder. Specify with or without a marked stressor, or with peripartum onset.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-004

Delusions, hallucinations, disorganized thinking (speech), grossly disorganized or abnormal motor behavior (including catatonia), and negative symptoms.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-003

6 months of continuous signs (prodromal and residual phases count), including at least 1 month of active-phase symptoms, with a decline in functioning.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-006

A mood disorder with psychotic features (major depressive or bipolar disorder), not schizoaffective disorder.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-005

Diminished emotional expression, avolition, alogia, anhedonia, asociality. They respond less to medication and predict poorer functional outcome.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-008

Delusional disorder: one or more delusions for 1 month or more, schizophrenia's other core symptoms absent, functioning largely intact apart from the delusion.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-007

Delusions or hallucinations for 2 weeks or more with no major mood episode, with mood episodes present for most of the total illness.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-009

New psychosis with visual, tactile or olfactory hallucinations, or onset after about age 40?

DSM-010

Schizotypal versus schizoid personality disorder?

DSM-011

Minimum duration of a hypomanic episode?

DSM-012

Minimum duration of a manic episode?

DSM-013

Bipolar I versus bipolar II?

DSM-014

Cyclothymic disorder?

DSM-015

Rapid cycling specifier?

DSM-016

A full manic episode appears during antidepressant treatment and outlasts the drug's effect. Diagnosis?

DSM-010

Schizotypal: odd beliefs, magical thinking, ideas of reference, social anxiety that doesn't ease. Schizoid: detached, restricted emotion, no odd beliefs.

DSM-5-TR (APA, 2022) Personality Disorders: Cluster A

DSM-009

Suspect a medical or substance cause (delirium, intoxication or withdrawal, seizures, tumor, medication). Medical evaluation first.

DSM-5-TR (APA, 2022) Schizophrenia Spectrum and Other Psychotic Diso...

DSM-012

1 week, or any duration if hospitalization is needed. Marked impairment, hospitalization or psychotic features define mania.

DSM-5-TR (APA, 2022) Bipolar and Related Disorders: Bipolar I Disorder

DSM-011

4 consecutive days, with a change others can observe. No marked impairment, no psychosis and no hospitalization (any of these makes it mania).

DSM-5-TR (APA, 2022) Bipolar and Related Disorders: Bipolar II Disorder

DSM-014

2 years (1 year in youth) of many hypomanic-type and depressive-type periods that never meet full episode criteria; never symptom-free for more than 2 months.

DSM-5-TR (APA, 2022) Bipolar and Related Disorders: Cyclothymic Disorder

DSM-013

Bipolar I: at least one manic episode (depression not required). Bipolar II: at least one hypomanic and one major depressive episode, and never mania.

DSM-5-TR (APA, 2022) Bipolar and Related Disorders: Bipolar I and II Dis...

DSM-016

It counts as a manic episode, so bipolar I. One or two symptoms such as irritability after starting an antidepressant do not.

DSM-5-TR (APA, 2022) Bipolar and Related Disorders: Bipolar I Disorder

DSM-015

4 or more mood episodes (manic, hypomanic or depressive) within 12 months.

DSM-5-TR (APA, 2022) Bipolar and Related Disorders: specifiers

DSM-017

Major depressive episode threshold?

DSM-018

Persistent depressive disorder?

DSM-019

Full major depression criteria are met right after a job loss. Diagnosis?

DSM-020

Normal grief versus a major depressive episode?

DSM-021

Prolonged grief disorder timing?

DSM-022

Peripartum onset specifier?

DSM-023

Premenstrual dysphoric disorder: how is it confirmed?

DSM-024

Disruptive mood dysregulation disorder (DMDD)?

DSM-018

Depressed mood most days for 2 years (1 year in youth), never symptom-free for more than 2 months. Major depressive episodes may occur within it.

DSM-5-TR (APA, 2022) Depressive Disorders: Persistent Depressive Disor...

DSM-017

5 or more of 9 symptoms for 2 weeks, including depressed mood or loss of interest, with distress or impairment. In youth, mood can be irritable.

DSM-5-TR (APA, 2022) Depressive Disorders: Major Depressive Disorder

DSM-020

Grief: waves of longing tied to reminders, self-esteem intact. Depression: pervasive low mood, worthlessness, self-criticism. DSM-5 removed the bereavement exclusion.

DSM-5-TR (APA, 2022) Depressive Disorders: Major Depressive Disorder (...

DSM-019

Major depressive disorder. Adjustment disorder is a residual category used only when no other disorder's criteria are met.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: Adjustm...

DSM-022

Mood episode onset during pregnancy or within 4 weeks after delivery.

DSM-5-TR (APA, 2022) Depressive Disorders: specifiers

DSM-021

At least 12 months after the death for adults (6 months for children and adolescents), with intense yearning or preoccupation most days for the past month. New in DSM-5-TR.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: Prolonge...

DSM-024

Severe outbursts about 3 or more times a week plus persistent irritability between them, 12 months or more, in 2 or more settings, onset before age 10.

DSM-5-TR (APA, 2022) Depressive Disorders: Disruptive Mood Dysregulat...

DSM-023

Prospective daily symptom ratings for at least 2 symptomatic cycles; provisional until then.

DSM-5-TR (APA, 2022) Depressive Disorders: Premenstrual Dysphoric Dis...

DSM-025

DMDD and ODD criteria both met?

DSM-026

Chronic, non-episodic irritability in a child versus distinct episodes of elevated mood?

DSM-027

Oppositional defiant disorder?

DSM-028

Conduct disorder?

DSM-029

Intermittent explosive disorder versus DMDD?

DSM-030

Age rules for antisocial personality disorder?

DSM-031

Can a personality disorder be diagnosed before 18?

DSM-032

Borderline personality disorder mood shifts versus bipolar episodes?

DSM-026

Chronic irritability points to DMDD; distinct episodes point to bipolar disorder. DMDD was added partly to curb pediatric bipolar overdiagnosis.

DSM-5-TR (APA, 2022) Depressive Disorders: Disruptive Mood Dysregulat...

DSM-025

Diagnose DMDD only. DMDD also cannot coexist with bipolar disorder or intermittent explosive disorder.

DSM-5-TR (APA, 2022) Depressive Disorders: Disruptive Mood Dysregulat...

DSM-028

Repeated violation of others' rights or major age norms: 3 or more behaviors in 12 months (1 in the past 6). Childhood-onset type if a symptom began before age 10.

DSM-5-TR (APA, 2022) Disruptive, Impulse-Control, and Conduct Disorder...

DSM-027

Angry or irritable mood, argumentative or defiant behavior, or vindictiveness: 4 or more symptoms for 6 months, with at least one person who is not a sibling.

DSM-5-TR (APA, 2022) Disruptive, Impulse-Control, and Conduct Disorder...

DSM-030

Age 18 or older, with evidence of conduct disorder beginning before age 15.

DSM-5-TR (APA, 2022) Personality Disorders: Antisocial Personality Disor...

DSM-029

IED: impulsive outbursts with normal mood between them, age 6 or older. DMDD: persistent irritability between outbursts, onset before 10.

DSM-5-TR (APA, 2022) Disruptive, Impulse-Control, and Conduct Disorder...

DSM-032

Borderline: shifts over hours, reactive to interpersonal stress. Bipolar: sustained episodes lasting days to weeks.

DSM-5-TR (APA, 2022) Personality Disorders: Borderline Personality Disor...

DSM-031

Yes, if traits have lasted at least 1 year, except antisocial personality disorder, which requires age 18.

DSM-5-TR (APA, 2022) Personality Disorders: general criteria

DSM-033

ADHD core requirements?

DSM-034

Inattentive symptoms appear only at home. ADHD?

DSM-035

Autism spectrum disorder requirements?

DSM-036

Social (pragmatic) communication disorder versus autism?

DSM-037

What sets severity in intellectual developmental disorder?

DSM-038

Tourette's disorder?

DSM-039

Enuresis thresholds?

DSM-040

Encopresis thresholds?

DSM-034

Criteria not met: several symptoms must occur in two or more settings. Gather teacher and parent ratings.

DSM-5-TR (APA, 2022) Neurodevelopmental Disorders: Attention-Deficit/...

DSM-033

6 or more symptoms (5 at age 17 and older) of inattention and/or hyperactivity-impulsivity for 6 months; several before age 12; several in 2 or more settings.

DSM-5-TR (APA, 2022) Neurodevelopmental Disorders: Attention-Deficit/...

DSM-036

Social communication deficits without restricted, repetitive behaviors or interests.

DSM-5-TR (APA, 2022) Neurodevelopmental Disorders: Social (Pragmatic...

DSM-035

All 3 social-communication deficits plus at least 2 of 4 restricted, repetitive behavior types, from early development. Severity (levels 1-3) rated separately for each domain.

DSM-5-TR (APA, 2022) Neurodevelopmental Disorders: Autism Spectrum...

DSM-038

Multiple motor tics and at least one vocal tic, more than 1 year since the first tic, onset before 18. Coprolalia occurs in only a minority.

DSM-5-TR (APA, 2022) Neurodevelopmental Disorders: Tic Disorders

DSM-037

Adaptive functioning (conceptual, social, practical domains), not the IQ score alone. Onset is in the developmental period.

DSM-5-TR (APA, 2022) Neurodevelopmental Disorders: Intellectual Devel...

DSM-040

Age 4 or older; at least once a month for 3 months. Usually with constipation and overflow; rule out medical causes first.

DSM-5-TR (APA, 2022) Elimination Disorders: Encopresis

DSM-039

Age 5 or older; voiding twice a week for 3 months or with significant distress. Rule out medical causes. Secondary type follows at least 6 months dry.

DSM-5-TR (APA, 2022) Elimination Disorders: Enuresis

DSM-041

Generalized anxiety disorder?

DSM-042

Panic disorder?

DSM-043

Agoraphobia?

DSM-044

Social anxiety disorder in children: special rule?

DSM-045

Separation anxiety disorder duration?

DSM-046

Selective mutism?

DSM-047

OCD with absent insight and delusional beliefs about the obsessions. Psychosis?

DSM-048

OCD versus obsessive-compulsive personality disorder?

DSM-042

Recurrent unexpected panic attacks plus at least 1 month of worry about more attacks or avoidance. Attacks only when speaking in public suggest social anxiety.

DSM-5-TR (APA, 2022) Anxiety Disorders: Panic Disorder

DSM-041

Hard-to-control worry about many things, more days than not for 6 months, with 3 or more symptoms such as tension or poor sleep (1 in children).

DSM-5-TR (APA, 2022) Anxiety Disorders: Generalized Anxiety Disorder

DSM-044

The anxiety must occur in peer settings, not only with adults. Duration 6 months; a 'performance only' specifier exists.

DSM-5-TR (APA, 2022) Anxiety Disorders: Social Anxiety Disorder

DSM-043

Fear or avoidance of 2 or more of 5 situations (transit, open spaces, enclosed places, crowds or lines, outside home alone) because escape or help may be hard; 6 months.

DSM-5-TR (APA, 2022) Anxiety Disorders: Agoraphobia

DSM-046

Consistent failure to speak in some settings (such as school) despite speaking elsewhere, for 1 month, not just the first month of school, and not due to learning a new language.

DSM-5-TR (APA, 2022) Anxiety Disorders: Selective Mutism

DSM-045

At least 4 weeks in children and adolescents; typically 6 months in adults. It can be diagnosed in adults.

DSM-5-TR (APA, 2022) Anxiety Disorders: Separation Anxiety Disorder

DSM-048

OCD: unwanted (ego-dystonic) obsessions and rituals. OCPD: an ego-syntonic personality pattern of perfectionism, rigidity and control.

DSM-5-TR (APA, 2022) Obsessive-Compulsive and Related Disorders: diff...

DSM-047

No. It stays OCD with the 'absent insight/delusional beliefs' specifier.

DSM-5-TR (APA, 2022) Obsessive-Compulsive and Related Disorders: Obs...

DSM-049

Body dysmorphic disorder?

DSM-050

PTSD timing?

DSM-051

Acute stress disorder timing?

DSM-052

Adjustment disorder timing?

DSM-053

Does trauma exposure through TV or social media count for PTSD?

DSM-054

Reactive attachment disorder versus disinhibited social engagement disorder?

DSM-055

Somatic symptom disorder?

DSM-056

Functional neurological symptom disorder (conversion disorder)?

DSM-050

Symptoms last more than 1 month. 'With delayed expression' if full criteria are not met until at least 6 months after the event.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: Posttrau...

DSM-049

Preoccupation with appearance flaws others see as slight or absent, plus repetitive checking, grooming or comparing. Has a 'muscle dysmorphia' specifier.

DSM-5-TR (APA, 2022) Obsessive-Compulsive and Related Disorders: Bod...

DSM-052

Onset within 3 months of the stressor; symptoms resolve within 6 months after the stressor or its consequences end.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: Adjustm...

DSM-051

3 days to 1 month after the traumatic event. If symptoms last longer and meet criteria, the diagnosis becomes PTSD.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: Acute St...

DSM-054

Both require a history of insufficient care. RAD: withdrawn, rarely seeks comfort. DSED: overly familiar with strangers. Developmental age at least 9 months.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: RAD and...

DSM-053

Not unless it is work-related, such as responders repeatedly reviewing disturbing material.

DSM-5-TR (APA, 2022) Trauma- and Stressor-Related Disorders: Posttrau...

DSM-056

Motor or sensory symptoms inconsistent with recognized neurological disease; not intentional. A psychological stressor is not required.

DSM-5-TR (APA, 2022) Somatic Symptom and Related Disorders: Functio...

DSM-055

Distressing body symptoms plus excessive thoughts, feelings or behaviors about them, persisting (typically 6 months). A medical explanation does not rule it out.

DSM-5-TR (APA, 2022) Somatic Symptom and Related Disorders: Somatic...

DSM-057

Factitious disorder versus malingering?

DSM-058

Factitious disorder imposed on another: who receives the diagnosis?

DSM-059

Anorexia nervosa severity in adults is based on what?

DSM-060

Bulimia nervosa frequency threshold?

DSM-061

Binge-eating disorder versus bulimia?

DSM-062

ARFID versus anorexia nervosa?

DSM-063

Depersonalization/derealization disorder: key feature?

DSM-064

Insomnia disorder threshold and first-line treatment?

DSM-058

The perpetrator (often a caregiver), not the victim. It also raises a mandated-reporting duty.

DSM-5-TR (APA, 2022) Somatic Symptom and Related Disorders: Factitio...

DSM-057

Both are intentional. Factitious: internal motive (sick role), no obvious reward. Malingering: external incentive, and it is not a mental disorder.

DSM-5-TR (APA, 2022) Somatic Symptom and Related Disorders: Factitio...

DSM-060

Binge eating and compensatory behavior both at least once a week for 3 months, with self-worth unduly tied to shape or weight.

DSM-5-TR (APA, 2022) Feeding and Eating Disorders: Bulimia Nervosa

DSM-059

Body mass index, from mild (17 or above) to extreme (below 15). Amenorrhea is no longer required. Types: restricting or binge-eating/purging.

DSM-5-TR (APA, 2022) Feeding and Eating Disorders: Anorexia Nervosa

DSM-062

ARFID: restriction from sensory aversion, low interest or fear of choking or vomiting, with no body-image disturbance.

DSM-5-TR (APA, 2022) Feeding and Eating Disorders: Avoidant/Restrictiv...

DSM-061

Weekly binges for 3 months with marked distress but no regular compensatory behavior. Most common eating disorder in U.S. adults.

DSM-5-TR (APA, 2022) Feeding and Eating Disorders: Binge-Eating Disor...

DSM-064

Trouble falling or staying asleep at least 3 nights a week for 3 months despite opportunity. First-line treatment is CBT for insomnia.

Qaseem, A., et al. (2016). Management of chronic insomnia disorder in a...

DSM-063

Feelings of detachment or unreality with reality testing intact, which separates it from psychosis.

DSM-5-TR (APA, 2022) Dissociative Disorders: Depersonalization/Derealiz...

DSM-065

Gender dysphoria?

DSM-066

Delirium hallmarks?

DSM-067

Major versus mild neurocognitive disorder?

DSM-068

Neurocognitive disorder with Lewy bodies: clues?

DSM-069

Frontotemporal neurocognitive disorder: clues?

DSM-070

Vascular neurocognitive disorder: clue?

DSM-071

Reversible contributors to rule out before assuming dementia?

DSM-072

Substance use disorder severity levels?

DSM-066

Acute onset (hours to days), fluctuating course, disturbed attention and awareness, and a physiological cause. A medical emergency.

DSM-5-TR (APA, 2022) Neurocognitive Disorders: Delirium

DSM-065

Marked incongruence between experienced and assigned gender for 6 months or more, with distress or impairment. Gender diversity itself is not a disorder.

DSM-5-TR (APA, 2022) Gender Dysphoria

DSM-068

Fluctuating attention, recurrent detailed visual hallucinations, parkinsonism after cognitive decline, REM sleep behavior disorder; severe antipsychotic sensitivity.

DSM-5-TR (APA, 2022) Neurocognitive Disorders: NCD With Lewy Bodies

DSM-067

Major: decline interferes with independence (needs help with complex tasks such as bills or medications). Mild: independence preserved, perhaps with extra effort.

DSM-5-TR (APA, 2022) Neurocognitive Disorders: Major and Mild Neuroco...

DSM-070

Decline linked in time to strokes or other cerebrovascular events, often stepwise, with attention and executive deficits prominent.

DSM-5-TR (APA, 2022) Neurocognitive Disorders: Vascular NCD

DSM-069

Earlier onset (often 50s-60s); prominent behavior change (disinhibition, apathy, loss of empathy) or language decline; memory relatively spared early.

DSM-5-TR (APA, 2022) Neurocognitive Disorders: Frontotemporal NCD

DSM-072

Mild 2-3, moderate 4-5, severe 6 or more of 11 criteria within 12 months.

DSM-5-TR (APA, 2022) Substance-Related and Addictive Disorders: subst...

DSM-071

Delirium, depression, medication effects, B12 deficiency, hypothyroidism, and normal pressure hydrocephalus (gait change, incontinence, confusion).

Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry...

DSM-073

Tolerance and withdrawal from a medication taken as prescribed?

DSM-074

SUD remission specifiers?

DSM-075

Which withdrawal syndromes can be fatal?

DSM-076

Which substance classes have no DSM withdrawal syndrome?

DSM-077

The only behavioral addiction in DSM-5-TR Section II?

DSM-078

Cultural Formulation Interview?

DSM-079

Z codes ('other conditions that may be a focus of clinical attention')?

DSM-080

'Other specified' versus 'unspecified' diagnosis?

DSM-074

Early remission: 3 to under 12 months with no criteria except craving. Sustained remission: 12 months or more.

DSM-5-TR (APA, 2022) Substance-Related and Addictive Disorders: specif...

DSM-073

They do not count toward a substance use disorder when the medication is used as prescribed under medical supervision.

DSM-5-TR (APA, 2022) Substance-Related and Addictive Disorders: subst...

DSM-076

Hallucinogens, PCP and inhalants. Cannabis and caffeine withdrawal are recognized.

DSM-5-TR (APA, 2022) Substance-Related and Addictive Disorders: withd...

DSM-075

Alcohol and sedative-hypnotic (benzodiazepine) withdrawal: seizures and delirium. Opioid withdrawal is rarely fatal, but overdose risk afterward is high.

DSM-5-TR (APA, 2022) Substance-Related and Addictive Disorders: withd...

DSM-078

A 16-question interview in Section III, usable with any client, to understand the problem in the client's own cultural terms.

DSM-5-TR (APA, 2022) Section III: Cultural Formulation

DSM-077

Gambling disorder. Internet gaming disorder is a Section III condition for further study.

DSM-5-TR (APA, 2022) Substance-Related and Addictive Disorders: Gam...

DSM-080

Other specified: the clinician states why criteria are not met. Unspecified: the reason is not given, for example when information is limited.

DSM-5-TR (APA, 2022) Section I: use of the manual

DSM-079

Not mental disorders. They record issues such as housing problems, relational problems or maltreatment. DSM-5-TR added codes for suicidal behavior and NSSI.

DSM-5-TR (APA, 2022) Other Conditions That May Be a Focus of Clinical...